

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Randomized, Open-label, Phase 3 Study of MK-2870 vs Chemotherapy (Docetaxel or Pemetrexed) in Previously Treated Advanced or Metastatic Nonsquamous Non-small Cell Lung Cancer (NSCLC) With EGFR Mutations or Other Genomic Alterations
Short Title:	Sacituzumab Tirumotecan (MK-2870) Versus Chemotherapy in Previously Treated Advanced or Metastatic Nonsquamous Non-small Cell Lung Cancer (NSCLC) With EGFR Mutations or Other Genomic Alterations (MK-2870-004)
Protocol Number:	412610573360339005
NCT Number:	NCT06074588
Sponsor Name:	Merck
Investigational Product:	pemetrexed
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression-free Survival (PFS) of Participants with NSCLC with Epidermal Growth Factor Receptor (EGFR) Mutations: PFS is defined as the time from randomization to the first documented disease progression per RECIST 1.1 based on blinded independent central review (BICR) or death due to any cause, whichever occurs first. 2. Overall Survival (OS) of Participants with NSCLC with EGFR mutations: OS is defined as the time from randomization to death due to any cause.
Secondary Obj.:	1. PFS of All Participants with NSCLC 2. OS of All Participants with NSCLC 3. Objective Response Rate (ORR) of Participants with NSCLC with EGFR Mutations

2.2 Background & Rationale

The purpose of this study is to evaluate sacituzumab tirumotecan versus chemotherapy (docetaxel or pemetrexed) for the treatment of previously-treated non-small cell lung cancer (NSCLC) with exon 19del or exon 21 L858R EGFR mutations (hereafter referred to as EGFR mutations or EGFR-mutated) or any of the follow genomic alterations: ALK gene rearrangements, ROS1 rearrangements, BRAF V600E mutations, NTRK gene fusions, MET exon 14 skipping mutations, RET rearrangements, or less common EGFR point mutations of exon 20 S768I, exon 21 L861Q, or exon 18 G719X mutations. The primary hypotheses are that sacituzumab tirumotecan is: (1) superior to chemotherapy with respect to progression-free survival (PFS) per RECIST 1.1 as assessed by BICR in NSCLC with EGFR mutations; and (2) superior to chemotherapy with respect to overall survival (OS) in NSCLC with EGFR mutations.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	556

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histologically- or cytologically-documented advanced (Stage III not eligible for resection or curative radiation) or metastatic non-squamous NSCLC with specific mutations.
- * Documentation of locally assessed radiological disease progression while on or after last treatment based on Response Evaluation Criteria in Solid Tumors Version (RECIST) 1.1.
- * Participants with genome mutations must have received 1 or 2 prior lines of epidermal growth factor receptor tyrosine kinase inhibitor (EGFR TKI), including a third generation TKI for participants with a T790M mutation; and 1 platinum-based therapy after progression on or after EGFR TKI.
- * Measurable disease per RECIST 1.1 as assessed by the local site investigator.
- * Archival tumor tissue sample or newly obtained core, incisional, or excisional biopsy of a tumor lesion not previously irradiated has been provided
- * Participants who have AEs due to previous anticancer therapies must have recovered to Grade ?1 or baseline.
- * Participants who are hepatitis B surface antigen (HBsAg) positive are eligible if they have received HBV antiviral therapy for at least 4 weeks, and have undetectable HBV viral load prior to randomization.
- * Human immunodeficiency virus (HIV)-infected participants must have well controlled HIV on antiretroviral therapy.
- * Have an ECOG performance status of 0 or 1 within 3 days before randomization.

4.2 Exclusion Criteria

- * Has predominantly squamous cell histology NSCLC.
- * Has mixed tumor(s) with small cell elements.
- * Has active inflammatory bowel disease requiring immunosuppressive medication or previous history of inflammatory bowel disease.
- * Has Grade ?2 peripheral neuropathy.
- * Has history of documented severe dry eye syndrome, severe Meibomian gland disease and/or blepharitis, or corneal disease that prevents/delays corneal healing.
- * Has uncontrolled, significant cardiovascular disease or cerebrovascular disease.
- * Has an EGFR T790M mutation and has not received a third generation EGFR TKI (eg, osimertinib).
- * Received prior systemic anticancer therapy including investigational agents within 4 weeks or 5 half-lives (whichever is shorter) before randomization.
- * Received a live or live-attenuated vaccine within 30 days before the first dose of study intervention.
- * Completed palliative radiotherapy within 7 days of the first dose. Participants must have recovered from all radiation-related toxicities and not require corticosteroids.
- * Received radiation therapy to the lung that is >30 Gy within 6 months of the first dose of study intervention.
- * Received prior treatment with a trophoblast cell-surface antigen 2 (TROP2)-targeted antibody-drug conjugate (ADC).
- * Received prior treatment with a topoisomerase I-containing ADC.
- * Has received an investigational agent or has used an investigational device within 4 weeks prior to study intervention administration.
- * Known additional malignancy that is progressing or has required active treatment within the past 3 years.
- * Active infection requiring systemic therapy.

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- * History of noninfectious pneumonitis/ILD that required steroids or has current pneumonitis/ILD.
- * Has known active central nervous system metastases and/or carcinomatous meningitis. Participants with previously treated brain metastases may participate provided they are clinically stable, radiologically stable for at least 4 weeks and do not require glucocorticoids for at least 14 days prior to randomization.
- * HIV-infected participants with a history of Kaposi's sarcoma and/or Multicentric Castleman's Disease.
- * Concurrent active Hepatitis B (defined as HBsAg positive and/or detectable HBV DNA) and Hepatitis C virus (defined as anti-HCV Ab positive and detectable HCV RNA) infection.

11. Study Identifiers & Governance

Primary ID: 412610573360339005

Registry Number: NCT06074588

Sponsor: Merck

Study Title: Sacituzumab Tirumotecan (MK-2870) Versus Chemotherapy in Previously Treated Advanced or Metastatic Nonsquamous Non-small Cell Lung Cancer (NSCLC) With EGFR Mutations or Other Genomic Alterations (MK-2870-004)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____